

Bedwetting (Nocturnal Enuresis) - Male (Child)

Algorithm 1485 | Child (5-17 years) | Gender at birth: Male

Definition

- Qatar-localized UAT-only pediatric bedwetting pathway screening for diabetes, urinary disease, constipation, neurologic symptoms and safeguarding; not approved for production.
- Child patient.
- Male.

Initial assessment

1. [OPEN_TEXT] How old is the child?
2. [YES_NO] Was the child previously dry at night for 6+ months?
3. [YES_NO] Have home strategies already been tried?
4. [OPEN_TEXT] What is the exact age; was the child previously dry; and are there excessive thirst/urination, weight loss, pain, fever, constipation, weakness, gait change, genital symptoms, stress, punishment, abuse or safeguarding concerns?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Urine - Blood In

Is visible blood in urine the main concern rather than nighttime bedwetting?

Visible blood in urine belongs in the Urine - Blood In pathway.

1. Disposition level 100

Along with the wetting, is the child confused, difficult to wake, fainting or collapsing, having a seizure, severely short of breath, unable to keep fluids down and rapidly worsening, in immediate danger, or disclosing abuse while the suspected unsafe person is present?

Severe systemic illness, neurological deterioration or immediate safeguarding danger requires emergency assessment rather than a routine bedwetting pathway.

1. Disposition level 70

After emergency features are excluded, is the child under 5, or is there excessive thirst or urination, weight loss, pain or fever, new daytime wetting, constipation with significant symptoms, weakness or gait change, genital symptoms, punishment, suspected abuse, or another safeguarding concern?

Children under 5 require an age-appropriate pathway, while diabetes, urinary, neurological, genital and safeguarding features require prompt pediatric assessment rather than routine enuresis advice.

1. Disposition level 50

Has the child kept wetting the bed despite trying home strategies, or started wetting the bed again after being dry for more than 6 months?

NHS.UK bedwetting guidance recommends primary-care review for these situations - a return of bedwetting after a dry period can occasionally indicate an underlying medical or emotional cause.

1. Disposition level 50

Is the child age 5 or older with typical bedwetting and none of the features above?

NHS.UK guidance states bedwetting in young children is normal, and many children under 5 experience it.

Advice

Emergency pediatric or safeguarding response

- For critical illness or immediate danger, call Qatar emergency services on 999 now and follow the call-taker's instructions. Keep the child with a safe adult where possible. Do not confront a suspected unsafe person or disclose the child's account to them. The exact Qatar child-protection reporting and safe-callback workflow remains GOVERNANCE_REQUIRED for UAT. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: reduced responsiveness, collapse or seizure; breathing difficulty or rapid deterioration; immediate safeguarding danger.

Prompt pediatric assessment

- Arrange prompt pediatric clinical review through the Qatar pathway approved for this UAT environment. A child under 5 must use a separately approved age-appropriate pathway. Excessive thirst or urination, weight loss, fever, pain, weakness, gait change, genital symptoms or safeguarding concerns must not be managed as routine bedwetting. The exact non-emergency destination, safeguarding report and safe-callback process remain GOVERNANCE_REQUIRED.
- Call Back If: the child becomes drowsy, weak or rapidly worse; vomiting or inability to drink develops; immediate safety concern develops.

Routine bedwetting follow-up

- Arrange a primary-care appointment - a bedwetting alarm or medication may help if home strategies haven't worked.

- Call Back If: daytime wetting also develops; pain with urination develops.

Supportive bedwetting review

- Never punish or shame the child. Arrange pediatric primary-care review through the Qatar UAT pathway before alarms or medicine; assess daytime symptoms, constipation, diabetes risk and safeguarding. Maintain normal daytime hydration and regular toilet access without forced fluid restriction.
- Call Back If: excessive thirst, weight loss, pain, fever or weakness; new daytime wetting or safeguarding concern.

First aid

- For critical illness or immediate danger, call Qatar emergency services on 999 now and follow the call-taker's instructions. Keep the child with a safe adult where possible. Do not confront a suspected unsafe person or disclose the child's account to them. The exact Qatar child-protection reporting and safe-callback workflow remains GOVERNANCE_REQUIRED for UAT. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Bedwetting", <https://www.nhs.uk/conditions/bedwetting/> (page last reviewed 11 April 2023)

Provenance

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